



## THE PLAN

# Where you stand

An honest look at what exists today, what is missing, and what to fix first.

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Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS\_BASE.

**PREPARED FOR**

The Founder, Vytalix

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**VERSION**

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## An honest look at what exists today, what is missing, and what to fix first.

### In one minute

- Everything that exists today is an idea, a name, a logo, one prototype and one conversation that went nowhere. That is the true starting point.
- The business scores **11 out of 100**. Under 20 means a company that has not started trading.
- Ten problems are listed below. The first four block everything else and take about four weeks to clear.
- Ten opportunities are listed too. The best one is not the product. It is selling your own advice while the product is built.
- Nothing here is a criticism of the idea. The idea is sound. The company around it does not exist yet.

## What to do

| Action                                           | Who                   | By when    |
|--------------------------------------------------|-----------------------|------------|
| Agree in writing who owns MaternalLink           | You and David Agunede | Day 3      |
| Register the company after checking the name     | You                   | Day 10     |
| Withdraw the 2025 AI deck and lock the prototype | You                   | Day 3      |
| Book a code and security check of the prototype  | A developer you pay   | Day 14     |
| Re-score this document                           | You                   | 14 October |

## What was actually reviewed

| What                                       | Where it was                   | What it is                                                                                        | Verdict                                                                           |
|--------------------------------------------|--------------------------------|---------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------|
| The code repository                        | GitHub                         | One file with one line of text                                                                    | Empty                                                                             |
| Your written brief                         | This project                   | A clear, complete description of the company you want to build                                    | The strongest thing you had                                                       |
| "Black Maternal health" deck, October 2025 | Google Drive                   | 13 slides proposing AI that predicts pre-eclampsia, sepsis, bleeding and blood clots              | An idea only. No data, no model, no clinician, no permission to use patient data  |
| Ekiti State proposal, March 2026           | Email                          | Sent by David Agunede as "Founder and CEO, Maternal Link". £4.5m for 15,000 women, rising to £36m | A proposal with no reply. The price is far above anything comparable              |
| The prototype dashboard                    | maternal-health-psi.vercel.app | A working website with patients, alerts and a risk engine that suggests clinical actions          | Real, but nobody has checked who owns the code or whether it is secure            |
| Kemek Enterprise investor files            | Google Drive                   | Your separate property business, with its own 128-name investor list                              | Not Vytalix. It shows you can run a disciplined process. It also splits your time |
| Public record for "Vytalix"                | Web search                     | No company found. Similar names exist in the UK and the US                                        | The name is at risk until checked properly                                        |

Nothing else was found. No business plan, no financial model, no website, no brand, no contracts, no customers, no revenue, no grants, no partners, no clinicians, no policies, no insurance.

## The score, area by area

Scoring: 0 means nothing exists. 3 means an idea or a draft. 5 means a usable draft that nobody has tested. 7 means it works and there is proof. 10 means best in class.

| Area                                   | Score           | Why                                                                                                |
|----------------------------------------|-----------------|----------------------------------------------------------------------------------------------------|
| Company and governance                 | 0               | No registered company, no board, no shareholder agreement                                          |
| Name and brand                         | 1               | A logo exists. The name has not been checked and clashes with others                               |
| Business model and prices              | 1               | One price existed and it was wrong                                                                 |
| Advisory and consulting                | 1               | An idea. No offers, no collateral, no clients. The fastest money in the company                    |
| MaternaLink                            | 2               | A prototype and a concept. No clinician, no evidence, no plan for regulation                       |
| Clinical safety                        | 0               | No safety officer, no hazard log, no statement of what the product is for                          |
| Data protection and regulation         | 0               | Not registered with the ICO. No data protection assessment. No thinking about medical device rules |
| Who owns the intellectual property     | 0               | Nothing written down with David Agunede or with whoever wrote the code                             |
| Technology                             | 1               | One prototype of unknown quality. No domain, no email, no cloud account                            |
| E-learning                             | 0               | Nothing exists                                                                                     |
| Proof that customers want it           | 1               | One meeting in Nigeria. No UK conversations at all                                                 |
| Knowing the competition                | 1               | The Ekiti proposal did not mention that the state already has a funded programme                   |
| Partners                               | 1               | One ministry meeting. No agreements                                                                |
| Sales system                           | 0               | No contact list, no scripts, no proposals                                                          |
| Marketing                              | 0               | No website, no company page, nothing published                                                     |
| Money and accounts                     | 0               | No model, no budget, no bank account, no accountant                                                |
| Ready for investors                    | 1               | You have shown you can run a list. Vytalix has no deck, no data room, no tax clearance             |
| Team                                   | 1               | One founder and one informal collaborator with unclear roles                                       |
| Routines and reporting                 | 0               | None                                                                                               |
| Credibility: advisers, evidence, press | 0               | None                                                                                               |
| <b>Total</b>                           | <b>11 / 100</b> | Pre-formation                                                                                      |

## What you have, what you are building, what comes next

Say it in this order to investors. Never inflate the first column.

### What you have

- A full company plan
- A prototype (needs an audit)
- A past government conversation
- A logo and two websites

### Building now

- The company itself
- Ownership agreement
- First advisory clients
- A hospital trial

### Next

- First paying customer
- A signed pilot
- Advisers on board
- Pre-seed round

*Say it in this order to anyone who asks. Never inflate the first column.*

## What is strong and what is weak

**Strong.** A precise founder brief. A genuinely important problem with real political attention behind it in the UK. A live conversation in Nigeria and a working prototype, which most founders at this stage do not have. Proof that you can run an investor list properly.

**Weak.** No company, no ownership, no brand protection. Medical claims with nothing behind them. A price built on nothing. One founder doing everything, across two businesses. No web presence at all.

**The chances.** Selling your own advice pays within weeks. A re-scoped MaternaLink can be trialled without becoming a medical device. Grants suit this story. Women's health and impact investors are actively looking for something like it. Partnering with the Nigerian incumbent turns a threat into a route in.

**The threats.** A name clash with two US health-tech companies. Regulatory trouble if any prediction feature ships. A falling-out over who owns MaternaLink. Slow, political government sales in Nigeria. An investor finding all of this in ten minutes.

## The ten problems, worst first

| # | Problem                          | Why it matters                                             | What fixes it                                                        | By     |
|---|----------------------------------|------------------------------------------------------------|----------------------------------------------------------------------|--------|
| 1 | Nobody owns MaternaLink on paper | Blocks funding, contracts and trials. Risks a dispute      | Meet this week, agree heads of terms, then a solicitor drafts a deed | Day 7  |
| 2 | No company                       | Cannot contract, bank, apply for grants or take investment | Check the name, then register                                        | Day 10 |
| 3 | Medical claims with no evidence  | Regulatory and reputational risk. Fails any investor check | Withdraw the deck. Re-scope the product. Rewrite what it does        | Day 3  |
| 4 | The name may not be yours        | Two US health-tech companies use near-identical names      | Trademark searches, then file                                        | Day 20 |
| 5 | No money coming in               | Your time is the only funding                              | Launch three advisory offers, hold 40 conversations                  | Day 45 |
| 6 | The £300 per woman price         | Anyone who knows the market will see it is wrong           | Replace with the new price book                                      | Day 21 |

| #  | Problem                         | Why it matters                                                       | What fixes it                                   | By     |
|----|---------------------------------|----------------------------------------------------------------------|-------------------------------------------------|--------|
| 7  | No clinician anywhere near this | You cannot build maternity software without midwives and doctors     | Recruit four advisers                           | Day 60 |
| 8  | The prototype is unchecked      | May contain insecure code, other people's work, or real patient data | Pay for a code and security audit               | Day 14 |
| 9  | Nobody in the UK has been asked | Your home market is untested                                         | 15 interviews with midwives and maternity leads | Day 60 |
| 10 | Two businesses, one of you      | Everything waits for you                                             | Write down your time split. Get help with admin | Day 14 |

## The ten opportunities, best first

| #  | Opportunity                                         | Why it is good                                        | First step                                          |
|----|-----------------------------------------------------|-------------------------------------------------------|-----------------------------------------------------|
| 1  | Sell your own advice                                | Cash in weeks, references, credibility                | Publish three offers, start 40 conversations        |
| 2  | MaternaLink as a care-coordination service          | The product story, without the medical device problem | Write what it is for, get one clinician to agree    |
| 3  | Work with the Nigerian incumbent, not against       | Keeps the government relationship alive               | Reply to Ekiti offering to complement, not replace  |
| 4  | Grants                                              | £50,000 to £500,000 without giving away shares        | Check the Innovate UK and SBRI Healthcare calendars |
| 5  | Women's health and impact investors                 | Better terms and aligned expectations                 | Work the A-list after the blockers are cleared      |
| 6  | Publish one research report                         | Authority, press, inbound interest                    | Outline it in week five                             |
| 7  | Training for midwives and health workers            | Repeat income, low regulatory risk                    | Two pilot modules with an NGO                       |
| 8  | Consulting for companies entering the NHS or Africa | High margin, repeatable                               | Package two fixed-price sprints                     |
| 9  | A university research partnership                   | Turns the 2025 AI idea into fundable research         | Approach a maternal health research group           |
| 10 | Telecoms and insurers in Nigeria                    | Reach without government budgets                      | Keep on the partner list for year two               |

## What creates value, and what creates risk

| Brings money in now                                   | Builds credibility                                                 | Makes investors comfortable                                              | Creates real risk                                                                                          |
|-------------------------------------------------------|--------------------------------------------------------------------|--------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------|
| Advisory days, consulting projects, grant-funded work | Advisers, a published report, a real trial, a professional website | A registered company, clean ownership, sensible prices, a named pipeline | Prediction claims, health data without safeguards, using the prototype with real patients, unclear purpose |

## Words explained

| Word                              | What it means                                                                                            |
|-----------------------------------|----------------------------------------------------------------------------------------------------------|
| Heads of terms                    | A short written summary of what two people have agreed, before the lawyers write the full contract       |
| ICO                               | The Information Commissioner's Office, the UK regulator you must register with to handle personal data   |
| Data protection impact assessment | A written check of what could go wrong with people's data, required before you handle health information |
| Hazard log                        | A list a health software team keeps of everything that could harm a patient, and what stops it           |
| Medical device                    | Anything, including software, whose purpose is to diagnose, predict or treat. Tightly regulated          |